

Patient: Olivia Martinez

Date of Birth: 03/05/1978 - Date of Service: 12/22/2024 - Referred by: Dr. Lisa Johnson - Gender: Female - ID: MD987654 - Private Insurance

SCREENING TESTS

Breast Cancer

Olivia Martinez has undergone regular breast cancer screenings over the years:

- On 07-10-2020, a Unilateral mammogram was performed, and it was classified as BI-RADS 1, indicated by the ICD-10 code 12.31.
- On 09-15-2022, a Bilateral mammogram was conducted, and it was classified as BI-RADS 2, indicated by the ICD-10 code Z12.31.
- On 04-20-2023, a Breast ultrasound revealed no significant abnormalities, and the ICD-10 code Z12.39 was used.
- Recently, on 11-10-2023, Olivia underwent a routine mammogram, which showed no concerning findings

Impressions

Well-managed migraines and anxiety

Comments

Olivia Martinez is managing her chronic conditions well and actively engages in stress-reducing activities.

Additional Information

Olivia Martinez is a single marketing executive who practices mindfulness and meditation for self-care.

Family History

Olivia Martinez family history is unremarkable for major medical conditions.

Social History

Olivia Martinez is a health-conscious individual who enjoys cooking nutritious meals and attending yoga classes. She actively participates in charity events and volunteers at a local animal shelter. Non smoker

Treatment Plan

Olivia Martinez is advised to continue her current management plan for migraines and anxiety, including regular check-ups and medication.

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Treatment Plan

Olivia Martinez is advised to continue her current management plan for migraines and anxiety, including regular check-ups and medication.

Prognosis

Olivia Martinez's prognosis is favorable as her conditions are well-managed and stable.

Discussion

Olivia Martinez is a case of chronic conditions with good management and stable health. She is actively participating in her helathcare and maintains a balanced lifestyle.

RENAL / METABOLIC

Polycystic Ovary Syndrome:None

Test (verbatim)	Date	Result / Range / Notes
Creatinine, Urine- ICD-10 code: R80.9	November 15, 2023	Result: 68.2 mg/dL Reference range: 20-320 mg/dL Urine sample collected to assess protein levels and kidney function.
Estimated Glomerular Filtration Rate (eGFR) ICD-10 code: N18.9	November 12, 2023	Result: Previous eGFR: 92 mL/min/1.73m2 Present eGFR: 89 mL/min/1.73m2
Albumin Test:ICD-10 code: R80.9	November 12, 2023	Result: 4.5 mg/dL Reference range: 3.5-5.5 g/dL Blood sample collected to measure albumin levels in the blood.
MICROALB/CRET RATIO (CALCULATED) : 4.0	—	—

PAST SURGICAL HISTORY

Procedure Name	Date	Notes
Bone Density Scan	04/2018	osteopenia.Frax 12% major,2.5%hip
Breast Lumpectomy	2006	right
Colonoscopy	6/2019	Polyp seen
Partial throidectomy	1996	—

PROCEDURE HISTORY

Report date:2/15/2022
Specimen collected:2/10/2022

COLOGUARD (Final Result)

Test Result	Value	Normal Value
Cologuard	Negative	Negative

Interpretation: Negative Cologuard result indicates low likelihood of colorectal cancer (CRC) or advanced adenoma.

Negative predictive value > 99.9%. Chance of CRC < 1 in 1500. Chance of advanced adenoma < 5.3%.

Recommendation: Proceed with FIT-DNA screening in one year

FIT DNA Test, Stool (External)

Report date:4/15/2023

Component	Result	Agency	Specimen collected:
FIT DNA Test, Stool	Negative	Exact Sciences Laboratories	4/10/2023

Procedure:	Screening for colon cancer-Z12.11
Indications:	Diarrhea, Positive fecal immunochemical test
Providers:	Son T. Do, MD
Referring MD:	Jennifer M. Byrne, DO
Medicines:	Monitored Anesthesia Care
Complications:	No immediate complications

PRE-ANESTHESIA ASSESSMENT

Prior to the procedure, a History and Physical was performed, and patient medications and allergies were reviewed. The patient is competent. The risks and benefits of the procedure and the sedation options and risks were discussed with the patient. All questions were answered and informed consent was obtained. Patient identification and proposed procedure were verified by the physician in the pre-procedure area.

Mental Status Examination:

alert and oriented.

Airway Examination:

normal oropharyngeal airway and neck mobility.

Respiratory Examination:

clear to auscultation.

CV Examination:

normal.

Prophylactic Antibiotics:

not required.

Prior Anticoagulants:

none taken.

ASA Grade Assessment:

II - mild systemic disease.

Findings

This test was performed three days after the FIT-DNA stool test, and the findings are as follows :

Sigmoid colon significantly tortuous and restricted mobility. Mucosa blanched with insertion of scope. Scope required exchanging to adult endoscope. Despite attempts, sigmoid colon could not be traversed due to tight angulation and restricted mobility. Exam aborted.

Impression

- Tortuous colon. Sigmoid colon with tight angulation and restricted mobility, not safely traversed. No specimens collected.

Recommendation

- Patient has emergency contact number.
- Signs/symptoms of complications discussed.
- Resume previous diet.
- Written discharge instructions given.
- Repeat colonoscopy after further studies.
- Consider virtual colonoscopy.

CT Colonography

History: Screening evaluation carried out on Columbus Day 2024.

Technique

Multiple contiguous CT transverse images, supine and prone. CO2 insufflation via rectal tube. Oral contrast given. IV contrast not administered.

Findings

No significant colonic polyp or mass lesion evident. Diminutive polyps <6 mm may be difficult to characterize. Flat polyps may be difficult to characterize. Other findings: small renal calculi, renal cysts (3.5cm), stomach collapsed, small bowel loops unremarkable.

Impression

No significant colonic polyp or mass lesion.

Recommendation: Patient is scheduled for colonoscopy on 11/2/2024

During the colorectal Screening using flexible tool, multiple polyps were discovered, with one exhibiting high-grade dysplasia. She is currently under surveillance for potential recurrence. Patient undergoes for surgical procedure in which the entire colon (large intestine) is removed. This procedure is performed due to the medical conditions that severely affect the colon, In this case of advanced colorectal cancer, so surgery is performed to remove the cancerous tissue and prevent further spread on 11/21/2024. Patient is scheduled for colonoscopy on 12/01/2024

PHYSICAL FINDINGS

The following vitals were recorded on Martin Luther King Jr. Day 2024. .

Measurement	Value	Normal Range
BP – Sitting L	148/68 mmHg	100-120/60-80
Standing L	138/72 mmHg	—
BP Cuff size	Large	—
Pulse Sitting	71 bpm	50-100
Respiration Rate	20 per min	18-26
Temp – Oral	97.8 F	96-101
Height	73 in	64-74
Weight	226 lbs	121-205
BMI	29.8 kg/m2	—
Oxygen Sat	97%	93-100

Nuclear Comparison

This nuclear perfusion scan when compared with the latest nuclear perfusion scan dated 11/13/2020 shows that:

- 1. Ejection fraction has increased from 38% to 56%.
- 2. Systolic volume has decreased from 69 ml to 48 ml. Nuclear Comparison
- No changes from previous study.

Time	HR (beats/min)	SBP (mmHg)	DBP (mmHg)	Comments
Baseline	78	120	72	—
Inject (Radiotracer)	105	134	70	—
Recovery 3 Min	102	114	68	—
Recovery 5 Min	93	124	68	—

MedCare Diagnostics Laboratory

Comprehensive Clinical Testing Services

Laboratory Report

Name:	Olivia Martinez	DOB:	03/05/1978	Identification No:	MD987654
Gender:	Female	Date of Service:	12/22/2024	Referred by:	Dr. Lisa Johnson
Line of Business:	Private Insurance				



Test Ordered: HEMOGLOBIN A1C

Component	Result	Ref Range & Units	Collection Date	Result Date
Hemoglobin A1C	6.6%	4.0 - 6.0%	03/18/2024	03/20/2024

Comment:
The ADA recommends that most patients with type 1 and type 2 diabetes maintain an A1C level < 7%.

OPHTHALMOLOGY — Dr. Lisa Johnson's Family Care Clinic

Patient Info

Patient Name: Olivia Martinez

DOB: 03/05/1978

Gender: Female

Resulted: 11/30/2024

OD (Right Eye)	
Exam:	An examination was performed
External:	normal lid position, nasolacrimal, and orbital exam
Lid Margin:	quiet and normal.
Conjunctiva:	white and quiet.
Cornea:	1+ corneal arcus.
Anterior Chamber:	deep and quiet anterior chamber.
Iris:	normal iris without rubeosis.
Lens:	1+ nuclear sclerosis.
Optic Disc:	flat and normal disc.
CD ratio:	OD: CD ratio 0.3.
Vitreous:	Weiss' ring.
Vessels:	vessels with normal contour, caliber without variation.
Macula:	macula normal contour without heme, edema, drusen, or exudate no diabetic retinopathy
Periphery:	periphery normal appearance without retinal tears, breaks, holes, or masses.

OS (Left Eye)	
External:	normal lid position, nasolacrimal, and orbital exam. OS
Lid Margin:	quiet and normal.
Conjunctiva:	white and quiet.
Cornea:	1+ corneal arcus.
Anterior Chamber:	deep and quiet anterior chamber.
Iris:	normal iris without rubeosis.
Lens:	1+ nuclear sclerosis.
Optic Disc:	flat and normal disc.
CD ratio:	OS: CD ratio 0.3.
Vitreous:	Weiss' ring.
Vessels:	vessels with normal contour, caliber without variation. OS
Macula:	macula normal contour without heme, drusen, or exudate no diabetic retinopathy.
Periphery:	periphery normal appearance without retinal tears, breaks, holes, or masses.

Above results are verified by ophthalmologist

Ordering Provider: Dr. Lisa Johnson

Performing Location:

Dr. Lisa Johnson's Family Care Clinic 789 Elm

Address: Avenue

City, State, ZIP

Med Labs Lab General

Lab specimen#: 234239

Patient Information: Olivia Martinez

SEX: Female

DOB: 03/05/1978

SSN: 1200987654

Address: 5678 Maple Street

Report Status: Final

Results Patient Id:

200202 Phone#:

987-654-321

GYN History

Date of Specimen :11/18/2023 00:00			Date Specimen received :11/18/2023 00:00		
Physician: Johnson, Lisa			Information:		
Source: Reported :12/05/2023					
Test Name	Lab	Result	Flags	Reference Range	

CYYTO CERV VAG INTERP

Report CASE:

PATIENT: Olivia Martinez

PATIENT HISTORY: Asymptomatic; Routine Screening

PAP TYPE: Liquid-Based PAP smear (automated/manual screening) (1 slide)

SPECIMEN SOURCE: Cervical/Endocervical Screening, Performed by City Medical Center, 789 Main Street, City, State, ZIP

MEDLABS CYTOPATHOLOGY REPORT SPECIMEN NO.:

SPECIMEN ADEQUACY:

Satisfactory for evaluation: endocervical/transformation zone present.

GENERAL DIAGNOSTIC CATEGORIZATION:

Negative for intraepithelial lesion or malignancy

Cellular evidence of atrophy.

Final Diagnosis Performed by Jane K Smith CT(ASCP) Electronically

Note:

Performed at Medical Laboratories of Central City (City) unless otherwise noted.

Report Provided By: City Medical Associates, 456 Elm Street, City, State, ZIP

Patient Information: Olivia Martinez
SEX: Female
DOB: 03/05/1978
SSN: 1200987654
Address: 5678 Maple Street

PREOPERATIVE DIAGNOSES

- Menometrorrhagia.
- Enlarged uterus.
- Inability to use medical management due to a history of pulmonary embolus.
- Pathology pending.

ANESTHESIA

General endotracheal.

ESTIMATED BLOOD LOSS

150 cc.

COMPLICATIONS

None.

DESCRIPTION OF THE PROCEDURE

After obtaining informed consent, the patient was taken to the operating room where general endotracheal anesthesia was introduced. She was placed in the dorsal lithotomy position and prepped and draped in the usual sterile manner. The bladder was drained using a red Robinson catheter. Clear urine was obtained. The anterior and posterior lips of the cervix were grasped with two Lahey thyroid clamps. The cervicovaginal junction was circumscribed using cautery. The bladder was reflected away from the cervix and uterus using cautery and blunt dissection. The posterior cul-de-sac was entered sharply. A suture was placed, uniting the posterior cul-de-sac peritoneum to the posterior vaginal mucosa and tagged for identification..